

Ethics, Governance, and Risk Note

Main Risks

Priority	Risk	Mitigation
High	False Negative Prediction (Missed ICH)	Human-in-the-Loop review, continuous model evaluation, and conservative prioritization policy.
High	Over-reliance on AI Recommendations	AI supports, but never replaces, clinical judgement. Confidence scores and explainability are always displayed.
Medium	Workflow Disruption	Pilot deployment, continuous performance monitoring, and seamless PACS integration.

Harm Scenarios

Stakeholder	Potential Harm
Patient	Delayed diagnosis if intracranial hemorrhage is missed.
Radiologist	Incorrect prioritization may delay urgent reviews or increase unnecessary workload.
Hospital	Reduced clinician trust, workflow disruption, and regulatory exposure.
Product Provider	Reputational damage and loss of customer confidence.

Controls

Area	Control
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Guardrails	AI is used only for worklist prioritization. Final diagnosis always remains the responsibility of the radiologist.
Review Process	Clinical validation, pilot deployment, and continuous performance monitoring before large-scale rollout.
Escalation Path	Low-confidence cases receive earlier human review. AI prioritization is suspended if patient safety is compromised.
Logging & Auditability	AI predictions, radiologist overrides, feedback, and model versions are logged to support traceability, continuous improvement, and regulatory audits.

Launch Blockers

Product launch should be delayed or suspended if:

- The predefined launch thresholds for **sensitivity, false negative rate, inference time, clinician adoption, and system availability** (defined in the **Evaluation Strategy**) are not achieved.
- Clinical validation identifies unacceptable patient safety risks.
- PACS integration disrupts existing clinical workflows.
- Regulatory or organizational approvals are incomplete.
- Pilot deployment demonstrates low clinician trust or adoption.

These launch blockers ensure that **patient safety, clinician trust, and workflow stability** are validated before NeuroTriage AI progresses to routine clinical deployment.